

JUDGMENT SHEET
IN THE PESHAWAR HIGH COURT,
D.I.KHAN BENCH
(Judicial Department)

Cr.A No. 27-D of 2024 with Cr.M No 339-D/2024.

Dr. Hafiz Muhammad Tahir
vs
Muhammad Salam

Date of hearing:	<u>07.04.2026</u>
Date of Announcement	<u>12.05.2026</u>
For Appellant:	Mr. Ehsan Ul Haq Malik, Advocate.
For Respondent:	Muhammad Salam, (in person)
For State:	Mr. Ahmad Abu Hanifa, AAG.
Research conducted by:	Mr. Riaz Aslam Daavi, Senior Research Officer, PHC.

JUDGMENT

IJAZ ANWAR, J. Through the instant appeal under Section 17(2) of the Khyber Pakhtunkhwa Consumer Protection Act, 1997 (“Consumer Act”), the appellant, a dental surgeon, has impugned the judgment dated 06.07.2024 passed by the learned Consumer Protection Court, Dera Ismail Khan, whereby he was directed to refund half of the treatment cost, along with compensation of Rs. 30,000/- and counsel’s fee of Rs. 30,000/-, to the respondent on account of alleged deficient dental treatment. The appellant prays as follows:

It is, therefore; requested that in the light of above submissions, this Hon’ble Court may very

graciously be pleased to set aside the impugned judgment and order Dated: 06.07.2024 and further be pleased to remand the complaint to learned court to decide a fresh after giving opportunity the appellant to lead evidence and cross examine the respondent's witnesses. This Hon'ble Court may further be pleased to grant any other relief deem appropriate in the circumstances of the case."

2. The appellant contends that the impugned judgment is illegal, arbitrary, and the result of non-application of judicial mind, having been rendered in his absence. It is further asserted that the respondent, an elderly patient, was duly informed about the nature, risks, and limitations of the dental procedure, and that no guarantee of success could be extended in accordance with settled medical principles. The appellant maintains that the learned trial court, lacking technical expertise, proceeded to decide complex medical issues without obtaining expert opinion, thereby exceeding its jurisdiction and ignoring material facts, including limitation and the respondent's alleged failure to seek proper follow-up. Much emphasis was laid before the Court on the question of jurisdiction, contending that in the presence of the Khyber Pakhtunkhwa Healthcare Commission ("HCC"), the Consumer Court lacks jurisdiction to entertain allegations of medical negligence. Therefore, the impugned order may

kindly be set aside.

3. Counsel for the respondent, as well as the learned AAG, while rebutting the arguments, submitted that the respondent, being a patient of the appellant, had availed medical services from him, which fall within the definition of “services” under the Consumer Act. Therefore, the learned Consumer Protection Court rightly exercised its jurisdiction. It was further submitted that proper opportunity of hearing was afforded to the appellant, but he himself waived the said opportunity by remaining absent. Thus, the impugned judgment cannot be set aside on account of his own failure.

4. Arguments heard and record perused.

5. The following pivotal questions arise for determination before this Court:

- 1. Whether the Consumer Protection Court had jurisdiction to entertain and decide the complaint relating to alleged medical negligence in the presence of the Khyber Pakhtunkhwa Healthcare Commission?**
- 2. Whether the appellant was afforded a fair opportunity of hearing?**

6. As to question No 1, the Consumer protection Court is a statutory forum established by the

Government under the Consumer Act, in terms of section 11A,¹ through notification in the official Gazette for each district, to be presided over by a District and Sessions Judge or an Additional District and Sessions Judge. The jurisdiction of the Court is delineated under section 12,² whereby it is competent to entertain complaints within the local limits where the opposite party resides or carries on business, or where the cause of action wholly or partly arises. Complaints may be instituted in accordance with section 13 by a consumer, a recognized consumer association, or the Director or authorized officer. Upon receipt of a complaint, the Court follows the procedure prescribed under section 14, and is vested with powers of a civil court under the Code of Civil Procedure, 1908, including summoning witnesses, receiving evidence, and obtaining expert analysis where necessary. Certain definitions provided under

¹11A. Establishment of Consumer Courts. Government may, by notification in the official Gazette, establish a Consumer Court in each District of the Province to be presided by a District and Session Judge or Additional District and Sessions Judge.

²12. Jurisdiction of the Court. Subject to the provisions of this Act, the Court shall have jurisdiction to entertain complaints within the local limits of whose jurisdiction, (a) the opposite party or each of the opposite parties, where there are more than one, at the time of the institution of the complaint, actually and voluntarily resides or carries on business or personally works for gain; or (b) any of the opposite parties, where there are more than one, at the time of the institution of the complaint, actually and voluntarily resides, or carries on business, or personally works for gain provided that in such case either the permission of the Court is given, or the opposite parties who do not reside, or carry on business, or personally work for gain, as the case may be, acquiesce in such institution; and (c) the cause of action wholly or in part arises.

section 2 of the Consumer Act are material for determining the jurisdiction of the Consumer Protection Court. A “*consumer*” under section 2(c) means a person or entity who buys goods or hires services for consideration, whether paid, partly paid, or promised, and includes any user or beneficiary thereof, but excludes a person obtaining goods for resale or commercial purposes. “*Services*” under section 2(n) include facilities made available to users for consideration, such as banking, transport, medical treatment, education, electricity, and similar services, but exclude services rendered free of charge or under a contract of personal service. “*Defect*” under section 2(e) means any fault or shortcoming in goods relating to their quality, quantity, potency, purity, or required standard, while “*deficiency*” under section 2(f) means any shortcoming in the performance, quality, or nature of a service undertaken under a contract or otherwise. The Consumer Act further defines “*unfair trade practice*” under section 2(o) as any dishonest, false, or misleading conduct adopted in the sale or provision of goods or services, causing loss or injury to a consumer, including misrepresentation, overcharging, false advertisement, and deceptive practices. Thus,

where a consumer purchases goods or avails services and the goods are found defective or the services deficient, or where unfair trade practices are involved, the aggrieved person may invoke the jurisdiction of the Consumer Protection Court under section 12 by filing a complaint in terms of section 13. Upon adjudication, if the Court is satisfied that the goods suffer from defect or the services from deficiency, or that the allegations regarding unfair trade practice are proved, it may, in exercise of powers under section 15, grant appropriate relief, including removal of defects, replacement of goods, refund of price or charges, or award of compensation. Furthermore, under section 16, the Court is empowered to impose penalties, including fine and imprisonment, for infringement of consumer rights. Admittedly, a patient is a consumer who avails services from a medical practitioner for consideration, and in case of deficiency in such services, he may approach the Consumer Protection Court. This was the legal regime in vogue from the promulgation of the Consumer Act until the enactment of the *Khyber Pakhtunkhwa Health Care Commission Act, 2015* (“**HCC Act**”), which created a special forum of Health Care Commission (“**HCC**”) for

matters relating to medical negligence.

Z. Under the *Khyber Pakhtunkhwa Health Care Commission Act, 2015*, the Health Care Commission is a statutory body established under section 3 for the regulation of health care services in public and private sectors on sound physical and technical footings. Its composition is provided under section 4, consisting of ten members, including three official members and seven non-official members from the private sector, possessing technical, professional, medical, legal, financial, quality assurance, health management, or patient safety expertise. The HCC is entrusted with broad powers and functions under section 6, including registration and licensing of health care establishments, enforcement of minimum standards of patient and health staff safety, regulation of public and private health care services, grant, renewal, suspension and cancellation of licences, investigation into maladministration, malpractice and failure in provision of health care services, imposition of fines, monitoring of service performance, protection of patient rights, and devising mechanisms to deal with quackery. For invoking the jurisdiction of the Health Care Commission, certain definitions provided under

section 2 of the *HCC Act* are material. For instance, a “*healthcare establishment*” under section 2(i) includes hospitals, diagnostic centres, blood banks, medical and dental clinics, nursing homes, maternity homes, and other premises or conveyances used wholly or partly for providing healthcare services in the public or private sector and declared as such by the Commission. “*Health care services*” under section 2(j) mean services relating to diagnosis, treatment, or care of persons suffering from any physical or mental disease, injury, or disability, including medical, dental, or surgical care and other notified services. A “*health care service provider*” under section 2(k) includes the owner, manager, or person in charge of a healthcare establishment, as well as professionals registered with relevant regulatory councils. The Act further defines “*quackery*” under section 2(o) as the practice of unqualified or unlicensed persons, or even licensed persons or institutions acting beyond the scope of their licence. Thus, matters relating to healthcare establishments, provision of healthcare services, conduct of service providers, and instances of quackery fall within the regulatory and adjudicatory domain of the Health Care Commission. The Act

further gives the HCC special statutory force through sections 18, 19, 20 and 33, whereby it may impose fines, proceedings relating to private health care services are protected within the framework of the Act, ordinary court jurisdiction is barred except as provided, and the Act is given overriding effect over inconsistent laws.

8. The definitions of medical negligence, criminal negligence, maladministration, and malpractice, though not provided in the *Khyber Pakhtunkhwa Health Care Commission Act, 2015*, are elaborately defined in the *Khyber Pakhtunkhwa Health Care Commission (Complaint Management) Regulations, 2022*, and are material for invoking the jurisdiction of the Commission. “*Medical negligence*” under regulation 2(aa) means failure to exercise due diligence or breach of duty of care by a healthcare establishment or service provider resulting in damage to a patient, whereas “*criminal negligence*” under regulation 2(k) refers to reckless conduct where an obvious risk to life or safety is willfully ignored. “*Maladministration*” under regulation 2(x) encompasses poor or failed administration, including arbitrary, unlawful, or negligent acts, delay,

inefficiency, or abuse of power in discharge of duties, while “*malpractice*” under regulation 2(y) denotes improper, unskilled, unethical, or illegal professional conduct causing harm. Under regulation 3, the Commission maintains a complaint management system through the Directorate of Legal Affairs for receiving, inquiring into, and deciding complaints. In terms of regulation 5, an aggrieved person may file a complaint initially before the healthcare establishment and, if dissatisfied, before the Commission within the prescribed period, subject to fulfillment of procedural requirements. The scope of complaints, as provided in regulation 6, includes medical negligence, criminal negligence, maladministration, malpractice, quackery, and failure in provision of healthcare services. Upon receipt of a complaint, the competent authority conducts a preliminary inquiry under regulation 7 and, if maintainable, proceeds with a formal inquiry in accordance with regulation 8, ensuring observance of principles of fair trial and natural justice, including issuance of notices, recording of evidence, and seeking expert opinion where necessary. The Health Care Commission, through the Competent Authority/Inquiry Officer/Inquiry Committee, is

vested with wide powers under regulation 9, including powers akin to those of a civil court for summoning witnesses, recording evidence, and inspecting healthcare establishments, and may also pass interim orders under regulation 10 to safeguard public interest. Upon conclusion of inquiry, it may impose penalties under regulation 13, including fines, compensation, and recommendations for criminal proceedings, suspension or cancellation of licence. Thus, the Regulations provide a comprehensive quasi-judicial framework for adjudication of complaints relating to medical negligence and allied matters within the domain of the Commission.

9. The term “*deficiency*” under the Consumer Act is a general concept relating to any shortcoming in the standard, performance, quality, or nature of a service. Medical negligence, criminal negligence, maladministration, and malpractice under the *Khyber Pakhtunkhwa Healthcare Commission Regulations, 2022* may, in a broad sense, fall within deficiency where they result in poor or substandard healthcare service. However, these terms are more specific, elaborate, and sector-oriented, as they deal with breach of duty of care, reckless disregard for patient

safety, administrative failure, and professional misconduct. Thus, by carving out these specific categories from the general concept of deficiency, the subsequent special law has created a distinct field for the Health Care Commission to take cognizance of such healthcare-related matters.

10. Indeed, the Consumer Act is a special law enacted for the protection of consumers. However, section 3 thereof assumes significance while determining questions of interpretation and jurisdiction vis-à-vis the Khyber Pakhtunkhwa Healthcare Commission Act, 2015. The said provision makes it clear that the Consumer Act neither replaces nor overrides any other law for the time being in force; rather, it operates in addition thereto. For facility of reference, section 3 is reproduced below:

“3. Act not to supersede other law. The provisions of this Act shall be in addition to and not in derogation or supersession of any other law for the time being in force.”

From a bare reading of the aforesaid provision, it is evident that the legislature has not given the Consumer Act an overriding effect. Instead, the remedy provided thereunder is additional to the remedies already available under other laws. This position was affirmed by this Court in *M/s Neelab CNG Filling Station v.*

Managing Director, Sui Northern Gas Pipelines (PLD 2013 Peshawar 9), wherein it was held that, in view of section 3, the Civil Court may adjudicate the matter notwithstanding the availability of a remedy before the Consumer Court.

11. In contrast to the Consumer Act, the HCC Act contains an express overriding clause. The said Act is not merely a subsequent enactment, but is also a special statute specifically designed to regulate healthcare establishments and to deal with matters relating to healthcare services, medical negligence, criminal negligence, malpractice, maladministration, quackery and allied subjects. Thus, while the Consumer Act provides a general additional remedy in respect of consumer goods and services, the HCC Act creates a specialized statutory regime for healthcare-related complaints. Applying the principle *generalia specialibus non derogant*, matters falling within this specialized healthcare framework are to be dealt with by the forum created thereunder. Moreover, where two statutes appear to operate in an overlapping field, preference is to be accorded to the statute containing a non-obstante clause, as such clause reflects the express legislative intent to give overriding effect to that

enactment. For facility of reference, section 33 of the HCC Act is reproduced below:

33. Overriding effect. ----Notwithstanding anything to the contrary contained in any other law, the provisions of this Act shall have an overriding effect and the provisions of any such law to the extent of inconsistency to this Act shall cease to have effect."

A bare reading of the aforesaid provision shows that, unlike the Consumer Act, the HCC Act has been clothed with overriding effect. Consequently, in matters concerning medical negligence and healthcare services, the HCC Act, being later in time, more specific in subject matter, and fortified by a non-obstante clause, shall prevail over the Consumer Act to the extent of inconsistency. Similarly, the settled principle of statutory interpretation is that a special law prevails over a general law, and where two special laws conflict, the later enactment prevails over the earlier one.³

12. This Court has already recognized and affirmed the jurisdiction of the Khyber Pakhtunkhwa Health Care Commission in matters relating to medical negligence. *In Lady Doctor Nargis v. The State*⁴ in the following terms:

21. Hence, the law is clear:

(i) no FIR can be registered against a healthcare

³ Dr. Saiqa Yousaf v. The State (2024 PCr.LJ 1852)

⁴ WP NO 994-A/2023 decided on 17.09.2025 (2025 PHC 5369)

service provider or a healthcare institution on the allegations of malpractice or negligence except at the instance of the commission;

(ii) **any person aggrieved of any malpractice or negligence by a healthcare service provider or institution may file a complaint u/s 13 of 'the Act of 2015' before the KP Healthcare Commission;**

(iii) it is the duty and responsibility of the commission to expeditiously and diligently pursue and take the complaint to its legal and logical conclusion, which may also, after due process, include lodging of FIR under the relevant provisions of PPC (including section 322);

(iv) it therefore follows that any such FIR registered directly without first approaching the Healthcare Commission would be without lawful authority and liable to be quashed.

Although the said case pertained to initiation of criminal proceedings, this Court clearly held that in matters involving medical malpractice or negligence, the aggrieved person must first approach the HCC by filing a complaint under section 13 of the HCC Act. In *Dr. Samdana Wahab v. The State*,⁵ this Court held that allegations of medical negligence require expert and technical scrutiny and cannot be directly subjected to ordinary criminal process or inquiry by a Judicial Magistrate on the direction of an Ex-Officio Justice of Peace. The HCC is the specialized statutory forum for such matters, and the complainant must first approach it. Consequently, the order directing magisterial inquiry was set aside, while the HCC was directed to entertain the complaint afresh on merits despite

⁵ WP No. 6100-P/2023 decided on 22.04.2026 (2026 PHC 2507)

limitation. This Court, in *Professor Dr. NiamatUllah v. Izaz Khan* (PLD 2026 Peshawar 49), has categorically held that the Consumer Protection Court has no jurisdiction to determine matters of medical negligence or malpractice, as such questions require expert assessment by the Khyber Pakhtunkhwa Health Care Commission under section 13 of the HCC Act.

The relevant portion is reproduced below:

7. Turning to another aspect of the case whether the Consumer Protection Court is the appropriate forum to determine whether medical malpractice or negligence has occurred, it is well accepted that for a claim for medical/clinical negligence to be established, a medical practitioner has to be found to have breached a duty of care to a patient, who in turn suffers injury as a result of that breach. Demonstrating that a doctor has breached the duty of care is the first major hurdle in any negligence case but this is not always clear cut. There is certainly scope for genuine differences of opinion when it comes to diagnosis and treatment. The complainant's contention is that the Consumer Protection Court lacks the jurisdiction and it is the Commission that is to make an assessment in that regard in view of the standards envisaged under section 13 of the Khyber Pakhtunkhwa Health Care Commission Act, 2015. Needless to say, liability claims for defective services, as envisaged under Section 13 of the Act *ibid*, would similarly entail a breach of a duty to be determined by the commission under section 13, it reads:

13. Complaints.---(1) An aggrieved person may, within sixty days from the date of knowledge of the cause of action, file a complaint against a healthcare service provider or healthcare establishment by submitting an application in writing supported by an affidavit, national identity card number and address of the aggrieved person. (2) The Commission shall not entertain an anonymous or pseudonymous complaint against a private health care service provider or healthcare establishment. (3) The Commission shall investigate in a transparent manner, the complaints relating to quality of health services, health services or system and medical

negligence. (4) The Commission shall prescribe the procedure for the conduct of investigation to be carried out by the Commission under this Act.

Reliance in this respect is placed on **PLD 2019 Lahore 429** Dr. Riaz Qadeer Khan vs Presiding Officer, District Consumer Court, Sargodha and others, the relevant is:

For what has been discussed above, we are persuaded to hold that Healthcare Commission is the only competent forum to investigate into the allegations of maladministration or malpractice by a health service provider and the Consumer Court has no jurisdiction to adjudicate upon such matters. Resultantly, all these writ petitions are allowed and the complaints pending before the learned Consumer Court are directed to be returned for its presentation before the appropriate forum.

8. In view of the above provision of law, the respondent No.1/complainant has not filed any complaint rather approached the Consumer Protection Court, which has no jurisdiction to try such like complaints.

The ratio of the aforesaid judgment directly governs the present controversy; consequently, the complaint ought to have been presented before the HCC, being the competent statutory forum. The Honorable Lahore High Court, in *Dr. Riaz Qadeer Khan v. Presiding Officer, District Consumer Court, Sargodha (PLD 2019 Lahore 429)*, held that the Healthcare Commission is the only competent forum to investigate allegations of maladministration or malpractice by a healthcare service provider, and that the Consumer Court has no jurisdiction to adjudicate such matters. In *Dr. Saiqa Yousaf v. The State (2024 P Cr. LJ 1852 Islamabad)*, Honorable Islamabad High

Court held that where a special statutory mechanism exists to deal with medical negligence, such as the Healthcare Commission or PMDC, the same must be invoked first, and no criminal proceedings can be initiated without exhausting that remedy. The Court further held that determination of medical negligence requires expert opinion, and in the absence of such technical evidence, registration of FIR or continuation of criminal proceedings is without lawful justification. Thus, in light of the above-referred judgments, and keeping in view the overriding effect conferred upon the HCC Act under section 33, the answer to question No. 1 is that the Consumer Protection Court has no jurisdiction to entertain or decide complaints relating to alleged medical negligence in the presence of the Khyber Pakhtunkhwa Health Care Commission.

13. As to question No 2, the record reveals that the appellant submitted a reply to the complaint and even appeared in person before the Consumer Protection Court, as is evident from the relevant portion of the impugned judgment reproduced below.

“The respondent was served to appear in the court where after his assistant Shan Behram appeared and submitted a written reply on behalf of the respondent doctor Muhammad Tahir. The

respondent doctor Muhammad Tahir himself also appeared in the court on 13.06.2024 and submitted the chemotherapy report of his wife and stated that his wife is sick and is in critical condition where she is being treated in Islamabad, therefore, he would be appearing in the court, however, he be heard. He was heard....

The reply available on the record, as well as the impugned judgment, confirms that the impugned order was not passed without affording an opportunity of hearing to the appellant. Rather, the appellant submitted his reply and also appeared in person before the Consumer Protection Court. Thereafter, he himself failed to pursue the proceedings. In such circumstances, he cannot blame the Court, shift responsibility upon it, or claim any indulgence on account of his own omission. His conduct disentitled him from raising the plea that he was condemned unheard. Thus, the appellant's assertion that no opportunity of hearing was provided is misconceived and contrary to the record. Accordingly, question No. 2 is answered in the affirmative.

14. During the proceedings, this Court noticed that the appellant, while describing himself on the prescription slip as “Dental Surgeon and Orthodontist”, also mentioned the qualifications “B.D.S. (Pesh), Gold Medalist, M.A.C.S. (U.S.A.), D-

Orth (Manilla University), CIRD (Implantology) U.S.A., and M.Sc. (Endo), Riphah University”, but did not mention his PMDC⁶ registration number. Under the *Pakistan Medical and Dental Council Act, 2022*, registration with the Pakistan Medical and Dental Council (PMDC) is a mandatory statutory requirement for lawful medical and dental practice in Pakistan. Sections 37(2)⁷ of the Act provide that a valid registration certificate shall constitute the licence to practice medicine and dentistry in Pakistan, while section 40(1)⁸ expressly prohibits any person, other than a registered medical or dental practitioner, from practicing medicine or dentistry. Furthermore, section 42(2) stipulates that no medical certificate, prescription or advice shall be considered valid unless issued by a practitioner possessing valid registration. The Act further obligates every registered practitioner, under section 43, to comply with the **Code of Medical**

⁶Pakistan Medical and Dental Council constituted under section 3 of Pakistan Medical and Dental Council Act, 2022.

⁷ 37(2) The practitioner shall be deemed to hold valid registration and entitled to be known and eligible for privileges of a registered dental practitioner if his name is retained on the register after fulfillment of requirements of continuation of registration as laid down by the Council from time to time and provided he had paid the dues of the Council. The valid registration certificate shall be the licence to practice dentistry in Pakistan

⁸40. Penalty of practicing without registration (1) No person, other than a registered medical or dental practitioner, shall practice medicine or dentistry. (2) Any person who acts in contravention of the provisions of sub-section (1) shall be punishable with imprisonment for a term which may extend to two years, but shall not be less than six months, or with fine which may extend to two hundred thousand rupees, but shall not be less than one hundred thousand rupees, or with both.

Ethics and prohibits the use or display of qualifications not duly recognized and registered under the law.⁹ The same requirement is also reiterated in the PMDC Code of Ethics for medical and dental practitioners. These provisions, when read collectively, demonstrate that valid PMDC registration, disclosure of registration number, and adherence to recognized qualifications are not mere formalities but mandatory legal obligations intended to ensure transparency, accountability, professional competence, and protection of patient rights. Such omission cannot be treated as a mere irregularity, particularly when Regulation 8¹⁰ of the PMDC Code of Ethics mandates every medical or dental practitioner to mention the registration number in all prescriptions, certificates and receipts, and further prohibits display of any degree or diploma not registered with the Council. Upon verification from the PMDC website, the registration of the appellant

⁹ 43(2) No registered person shall use or publish in any way whatsoever any time, title, description or symbol indicating or calculated to lead persons to infer that he possesses any additional or other professional qualification unless the same has been conferred upon him by a legally constituted authority within or outside Pakistan and which is recognized under this Act.

¹⁰“Every medical or dental practitioner shall, in his clinic or place of practice, display a copy of valid registration certificate issued to him by the Council and refer his registration number in all his prescriptions, certificates, money receipts given to his patients.

(2) No medical or dental practitioner shall display suffixing to his name those degrees or diplomas which have not been registered by the Council...”

was found as No. 1658-D¹¹ in the name of *Mohammad Tahir son of Malik Mehraban*, showing BDS, Basic Dental Qualification, University of Peshawar, passing year 1987, whereas the additional qualifications and specialist description appearing on the prescription slip were not reflected therein. The matter, therefore, raises serious questions regarding non-disclosure of registration number, possible misdescription of professional status, display of unregistered qualifications, competence to perform the treatment, and compliance with ethical standards. Accordingly, the Khyber Pakhtunkhwa Health Care Commission shall examine whether the appellant was duly qualified and authorized to undertake the treatment in question, whether the clinic or place of practice was duly registered or licensed, whether the procedure was permissible at such place and within the registered competence of the appellant, whether necessary protocols, informed consent, record-keeping requirements and post-treatment obligations were observed, and whether the treatment adopted was in the best interest of the patient. The Healthcare Commission shall consider all aspects of the matter,

¹¹<https://pmdc.pk/>

including the observations made hereinabove, for fair, lawful, and just adjudication of the complaint.

15. Before parting with this judgment, and in view of the observations made in paragraph No. 14, this Court considers it necessary to emphasize that the HCC, has been vested with a comprehensive statutory mandate to regulate healthcare services, ensure patient safety, and enforce ethical and professional standards in both public and private healthcare establishments. Section 6 of the HCC Act expressly empowers the HCC to enforce minimum standards of healthcare, regulate healthcare establishments through inspection and oversight mechanisms, inquire into maladministration, malpractice, and failures in the provision of healthcare services, and devise effective mechanisms to curb quackery in all its forms and manifestations. The HCC Act further authorizes the HCC to investigate complaints, conduct inspections, compel production of documents, and exercise powers analogous to those of a Civil Court. Equally, the *Pakistan Medical and Dental Council Code of Ethics* imposes mandatory obligations upon every medical and dental practitioner. Regulation 7 unequivocally prohibits the practice of medicine or dentistry without

valid registration with the Council, while Regulation 8 mandates that every practitioner shall display and mention his PMDC registration number on all prescriptions, certificates, and medical documents and shall not display or claim any qualification, diploma, or specialty not duly recognized and registered by the Council. The Code further obligates practitioners to maintain honesty, transparency, professional competence, and ethical conduct in all dealings with patients, and prohibits association with unregistered or unauthorized persons practicing medicine. Despite the existence of such elaborate statutory and ethical framework, it is a matter of serious concern that prescription slips are still being issued without mention of PMDC registration numbers, and qualifications not recognized or registered with PMDC continue to be displayed by practitioners. Such conduct not only amounts to a clear breach of the PMDC Code of Ethics but also undermines public confidence in the healthcare system and raises serious concerns regarding transparency, accountability, and professional competence. The continued prevalence of quackery and unauthorized medical practice further reflects a regulatory failure requiring immediate and

effective intervention by the HCC in exercise of its statutory functions under Sections 6, 13 and 14 of the HCC Act.

16. *The Khyber Pakhtunkhwa Healthcare Commission (Complaints Management) Regulations, 2022* further obligate the Commission to entertain and inquire into complaints relating to malpractice, maladministration, quackery, unethical conduct, failure to follow medical standards, documentary irregularities, and violations affecting patient rights. The Regulations specifically define “malpractice” as improper, illegal, unethical, or unskilled professional conduct and empower the Commission to initiate inquiries, conduct inspections, seek expert opinions, and pass speaking orders in accordance with principles of natural justice. Therefore, failure to disclose a valid PMDC registration number, misrepresentation of qualifications, permitting unauthorized persons to practice medicine, or allowing healthcare establishments to function in violation of prescribed standards are all matters squarely falling within the jurisdiction and regulatory competence of the HCC.

17. This Court is also constrained to observe that irrational prescribing practices and excessive dependence upon branded medicines continue to place unnecessary financial burden upon patients, particularly those belonging to economically weaker segments of society. Although no absolute statutory prohibition exists against prescribing branded medicines, the PMDC Code of Ethics expressly requires rational use of drugs and obligates practitioners to act in the best interests of patients free from commercial influence. Regulation 9¹² of the PMDC Code of Ethics provides that every medical or dental practitioner shall adopt good and rational practices while prescribing drugs. Regulation 29(2)¹³ further provides that financial or commercial interests in organizations providing healthcare, or in pharmaceutical or other biomedical companies, must not affect the manner in which patients are prescribed, treated, or referred. If both provisions are read together, their combined object is to ensure that prescription of medicines is guided by therapeutic

¹²9. Rational use of drugs.- (1) Every Medical or dental practitioner shall adopt practice with good and rational practices to prescribe drugs.

¹³29(2) Financial or commercial interests in organizations providing health care or in pharmaceutical or other biomedical companies must not affect the way that patients are prescribed, treated or referred.

need, patient welfare, and rational medical judgment, and not by commercial influence, brand promotion, or pharmaceutical inducement. The expression “drugs” in Regulation 9, therefore, is to be understood in its pharmacological and therapeutic sense, i.e., by reference to the generic substance or composition of the medicine, rather than the branded name under which it is marketed. Thus, the spirit of Regulations 9 and 29(2) promotes generic prescribing as a rational, transparent, affordable, and patient-centered practice. Its object is to ensure therapeutic equivalence, affordability, transparency, and protection of patient welfare, rather than encouraging the prescription of particular branded medicines under the influence of commercial or financial considerations. In modern healthcare systems, generic prescribing is recognized internationally as a sound and ethical practice aimed at ensuring affordability, accessibility, transparency, and equitable healthcare delivery. The establishment of generic medicine pharmacies in every District Headquarters Hospital and public healthcare institution would reduce the cost of treatment for ordinary citizens and make essential medicines more affordable and accessible, particularly for the poor. It

would also discourage unnecessary expenditure on pharmaceutical branding, promotional practices, and excessive packaging. Such packaging, admittedly, involves the use of plastic, paper, and other synthetic materials, which contributes to environmental degradation. Promotion of generic medicine pharmacies would, therefore, serve the twin objects of public welfare and environmental sustainability.

18. Accordingly, this Court expects that the HCC shall discharge its statutory obligations in their true letter and spirit by ensuring strict enforcement of the PMDC Code of Ethics, taking effective action against quackery and unethical medical practices, ensuring that only duly registered qualifications are displayed by practitioners, compelling compliance regarding mention of PMDC registration numbers on all prescriptions and medical documents, and promoting rational and patient-centric prescribing practices, including encouragement of generic prescribing wherever clinically appropriate. Simultaneously, the Healthcare Commission may, within its regulatory competence, take appropriate measures to encourage rational prescribing practices, including issuance of guidelines, awareness initiatives,

and institutional mechanisms, so as to safeguard patient interests and promote equitable access to essential medicines. The Commission shall further strengthen inspection, monitoring, complaint redressal, and regulatory enforcement mechanisms so as to safeguard patient rights, maintain professional integrity, and restore public confidence in the healthcare system. The Additional Registrar (Judicial) of this Court is directed to transmit copies of this judgment to the worthy Chief Secretary, the worthy Secretary Health, and the Health Care Commission of KPK for compliance with the observations and directions contained in paragraphs Nos. 14 to 18 of this judgment.

19. In view of the foregoing reasons, the instant appeal is allowed and the impugned judgment is set aside on the ground that the Consumer Protection Court lacks jurisdiction in matters relating to medical negligence, particularly in the presence of the HCC as the competent statutory forum. The complaint shall be returned to the complainant, who may present the same before the HCC. The HCC is directed to decide the complaint, in accordance with law, within thirty days of its receipt, while keeping in view the

observations made in paragraph No. 14 of this judgment.

Announced
12.05.2026

**Senior
Puisne Judge**

Judge

DB Mr. Justice Ijaz Anwar, and Mr. Justice Sadiq Ali, Hon’ble Judges